

METRO GENERAL HOSPITAL - SURGICAL SERVICES

TRAUMACAREUNIT | WOUNDMAPPING&REPAIRLOG

PATIENT: DOE, JOHN

ID: JD-021426

DATE: 02/14/2026

I. CLINICAL PRESENTATION (PRE-OPERATIVE)

Patient arrived via EMS trauma transport. Examination reveals an **extensive 12cm stellate (star-shaped) laceration** located on the left malar region extending to the mandibular border. Wound edges are jagged and irregular, consistent with high-velocity tempered glass intrusion from the B-pillar side-window shattering.

II. FOREIGN BODY REMOVAL

Visual and manual exploration of the wound bed revealed multiple foreign bodies. Three (3) shards of tempered automotive glass, ranging from 2mm to 5mm, were recovered from the subcutaneous adipose layer. Significant debridement of contaminated tissue was required to ensure wound viability.

III. MULTI-LAYER CLOSURE TECHNIQUE

Due to the depth and irregularity of the wound, a single-layer closure was deemed insufficient. A **Multi-Layer Surgical Repair** was initiated:

- **Subcutaneous Layer:** 12 Interrupted Vicryl 4-0 sutures utilized for deep tissue apposition and tension reduction.
- **Epidermal Layer:** 30 Prolene 6-0 sutures utilized for fine skin-edge alignment.
- **Total Suture Count:** 42 sutures.

IMPRESSION: High risk for hypertrophic scarring and permanent cosmetic disfigurement. Wound architecture suggests significant tension across the malar fold, likely resulting in a permanent visible deficit.

Electronically Verified by K. Miller, MD | Chief of Trauma Surgery

